

2. **Abuse.** Periodically, social scientific or clinical research or journalistic exposes uncover cases in which nursing homes residents have been physically or mentally or pharmaceutically (e.g., overuse of antipsychotic medication) abused.
3. **Accidents.** Especially in understaffed homes, serious resident accidents—for example, falls and burns—can occur.
4. **The high price of care.** In 2013 it was estimated that the median daily cost of a private room in a nursing home had reached \$230, that is, \$6,900 a month or almost \$84,000 per year. Assisted living care averaged about \$41,000 per year. Few individuals or families can afford such extraordinary costs. It has become quite common for individuals to intentionally almost deplete all their financial resources in order to qualify for Medicaid because Medicaid pays for

long-term care. Medicaid is the largest payer for long-term care, and paying for long-term care consumes nearly a third of Medicaid expenditures. This pattern sharply drives up the cost of Medicaid, which, as described in Chapter 14, is a major financial issue for state governments and the federal government.

HOSPICES

The term **hospice** refers to a philosophy of providing care and comfort to people in the dying process. As far back as the eleventh century, the word was used to identify guesthouses and places of shelter for sick and weary travelers. During the 1960s, British physician Dr. Cicely Saunders developed a modern approach to hospice that emphasized professional caregiving



IN THE FIELD

FOCUS ON A FREE HEALTH CLINIC

The Bradley Free Clinic in Roanoke, Virginia, was established in 1974 with \$250 in seed money, one volunteer physician, one volunteer nurse, and the free rental of the first floor of an old house (donated by the adjacent church). With the dedication of a small group of concerned citizens and the energy and enthusiasm of a barely paid director, the clinic was able to offer free health services to the local medically indigent two nights per week. The commitment of the director and volunteers was noticed in the medical community, by other lay volunteers, and by local governments—important factors in the clinic's subsequent support.

The clinic each year provides more than 20,000 patient visits at no charge, prescribes more than 50,000 medications (almost all of which are given at no charge out of the clinic's own pharmacy—filled mostly with drugs donated by pharmaceutical companies and local physicians), offers extensive dental services (mostly out of its own fully modern dental

operatories—supplied by donations of dental equipment companies and local dentists), provides countless hours of mental health counseling (by local professionals volunteering their time), and performs basic lab tests (in its own small laboratory of mostly donated equipment). The clinic now sits in its own medical building (purchased with funds from a \$1 million donation by a local philanthropist). The estimated value of annual services provided is more than \$4 million (Weiss, 2006).

Supplementing a small paid staff are a host of volunteers: 100 physicians who volunteer some time at the clinic, plus 50 specialists who accept free referrals of clinic patients; 60 nurses; 30 pharmacists; 12 laboratory technicians; 20 dentists; 15 dental assistants; a dozen mental health counselors; and scores of lay volunteers. Care is provided at no charge in an atmosphere of respect for the dignity of each patient. By 2011 the Bradley Free Clinic had provided care valued at more than \$55 million for more than 335,000 patients.

and use of modern pain management techniques to compassionately care for the dying. She worked with others in establishing St. Christopher's Hospice near London, a hospice that significantly influenced the creation of other hospices around the world. The first hospice in the United States was established in New Haven, Connecticut, in 1974.

During the years of its development, hospice has evolved from offering services only in its own locations to offering services within hospitals, nursing homes, and especially in the patient's own home. Surveys indicate that a very large percentage of Americans would prefer to die in their own home, and about 70 percent of hospice patients do so. There are approximately 5,300 hospice providers in the United States today. Hospices range in size from small, all-volunteer staffs who provide services for fewer than 50 persons per year to large, multi-hospice providers serving several thousand persons each day. In 2012, an estimated 1.54 million persons received hospice care, including just over 1 million who died during the year. Others were still in hospice care at the end of the year or had returned to curative care (National Hospice and Palliative Care

Organization, 2013). About 45 percent of people who died in 2011 were under the care of a hospice at the time of death.

Benefits of Hospice. Hospice services are available only to patients who have been attested by two physicians to be in the last six months of life. Hospices regard the dying process as being a normal part of life. They attempt to make patients as comfortable and pain free during these months as possible; they do not do anything to hasten or postpone death. The average length of stay in hospice is 70 days. Contrary to a popular perception, hospice patients on average live one month longer than comparable patients who do not receive hospice care.

Hospice staff may include physicians, nurses, social workers, counselors, home health care aides, clergy, therapists (physical, occupational, massage, recreational, music, art, pet, etc.), dietitians, and volunteers. Following are among the services offered to patients and their families (The Hospice Foundation, 2013):

- Knowledge about medical care and the dying process by specially trained professionals, volunteers, and families working in a team approach

- Addressing all symptoms of diseases but with a special emphasis on controlling a patient's pain and discomfort (i.e., **palliative care**) and treating patients with concern and dignity
- Dealing with the emotional, social, and spiritual needs of patients and families
- Offering bereavement and counseling services to families before and after a patient's death

Payment for hospice services is covered by Medicare, Medicaid, and many private health insurance policies. Hospices work with uninsured patients to determine if they might qualify for any insurance or financial assistance. Often, patients do, and if not, many hospices will accept them anyway. The Medicare and Medicaid Hospice Benefit has been available for individuals whose condition requires reasonable medical and support services but no longer would benefit by curative services. If a patient lives longer than six months, payment continues as long as the attending physician still considers the patient to be terminally ill.

Beginning in the 1980s, some hospices converted to an "open access" model in which terminally ill patients could continue to receive chemotherapy and other curative treatments even while being under hospice care. In part this model was inspired by some AIDS patients who wanted to be in hospice care but also wanted to continue trying other medical options. The Affordable Care Act passed in 2010 directed the State Children's Health Care Program and Medicaid to immediately cover simultaneous medical care and hospice care for children with terminal illnesses. The law also calls for a three-year pilot program to test simultaneous care for Medicare patients (O'Reilly, 2010).

Several factors have contributed to the growth of hospices in the United States. These include the increasing number of older persons who are experiencing and dying from chronic and painful illnesses, the difficulties of patients and families having to contemplate death through a long period of dying, the increasing cultural value of "death with dignity," and

popular support for the humaneness of the hospice philosophy.

Concerns About the Future of Hospice.

While hospice has become an increasingly popular concept and program in the United States, there are pockets of resistance. Some individuals are uncomfortable with any approach that conflicts with doing everything possible to prolong the life of a patient. The acceptance of death and the dying process that are part of hospice is objectionable to them.

Some hospice enthusiasts strongly prefer the original model of hospice as "an antiestablishment, largely volunteer movement advocating a gentle death as an alternative to the medicalized death many people had come to dread" (Henig, 2005:3). The open-access model in which patients may be receiving various forms of high-technology care concurrently with hospice care seems a contradiction to the kind of serene environment on which hospice was grounded.

This concern is amplified by the fact that ownership of hospice is shifting rapidly from the nonprofit to the for-profit sector. The hospice movement was entirely nonprofit in its origin, and as late as 1983, all U.S. hospices were nonprofit owned or government owned. However, by 2001, one-third of U.S. hospices were owned by individuals or companies seeking to make a profit (often hospitals or home health care companies). By 2011, about 60 percent of all hospice ownership was for profit (34 percent nonprofit and 6 percent government). The shift toward for-profit ownership has stimulated higher costs and significantly increased Medicare spending for hospice. In 2011 the nation's fastest-growing for-profit hospice owner agreed to pay \$12.5 million to settle Medicare fraud claims, and in 2013 the nation's largest for-profit hospice owner was sued by the Department of Justice for tens of millions of dollars in Medicare fraud (billing for ineligible patients and inflated services). Discussion of profit margins, productivity adjustments, and market efficiencies seem to many to clash with the original ideals of hospice.